

Derbyshire Shared Care Pathology Guidelines

Hyperferritinaemia in adults

Purpose of Guideline

The management and referral criteria of patients with newly diagnosed elevated ferritin in adults.

Background

Ferritin is the storage protein for iron within the cells. Plasma ferritin correlates with intracellular levels and that of total body iron stores. It can increase very quickly (within 24hrs) if large doses of iron are given.

Ferritin is an acute phase protein.

During inflammation or infection there is a shift of iron being moved into the tissues and increased ferritin concentrations are seen.

Ferritin is normally assessed in relation to possible anaemia and an isolated raised level can be an unexpected finding.

Reference Ranges

Males	>16 years	30 - 400 µg/L
Females	>16 years to 60 years	15 - 150 µg/L
Females	>60 years	30 - 400 µg/L

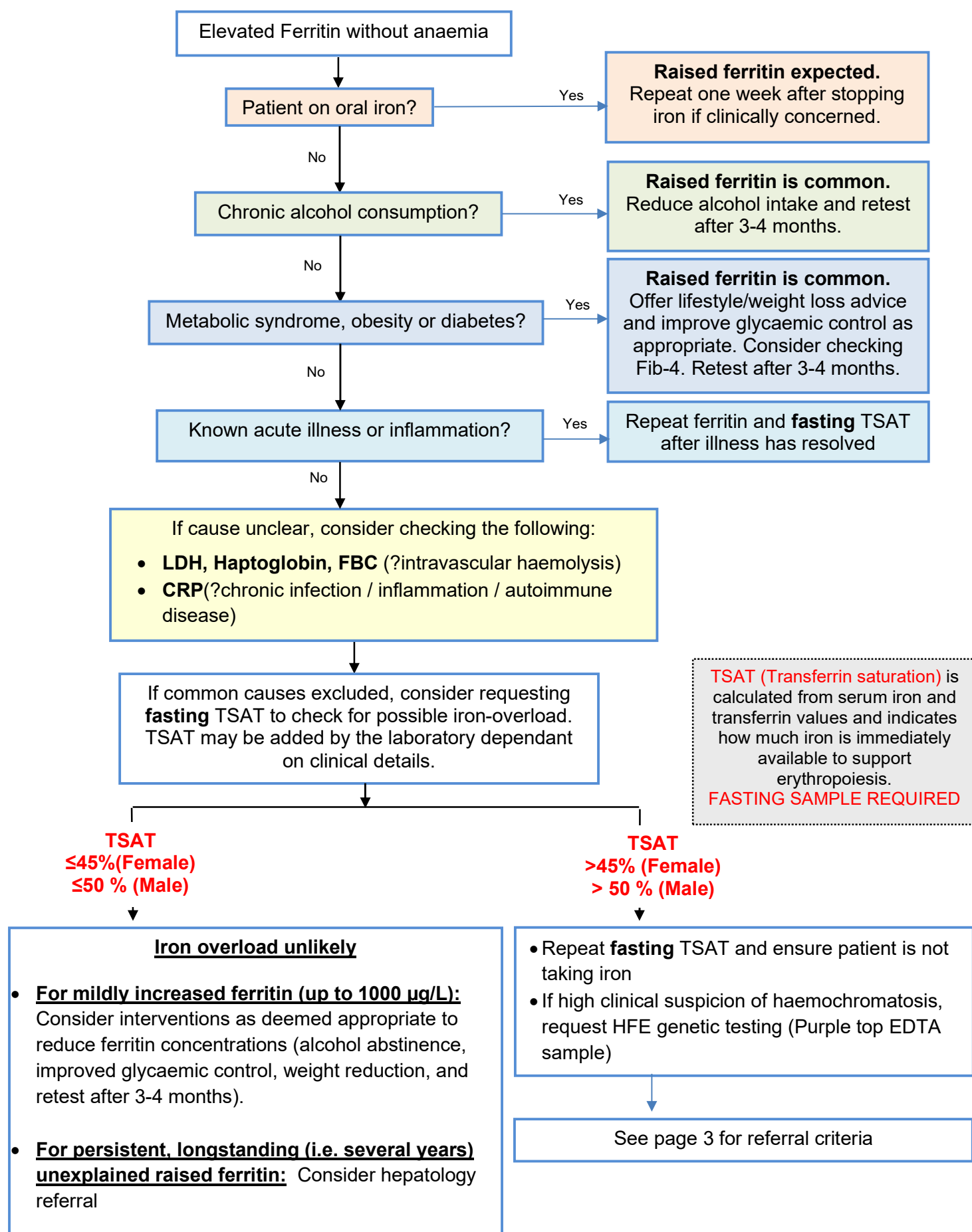
When to suspect a high ferritin?

Ferritin can be raised in a wide range of diseases and benign conditions as shown below. High levels with unexplained liver disease, infertility or fatigue may be suggestive of genetic haemochromatosis.

What are the major causes of hyperferritinaemia?

- Acute/Chronic inflammation
- Recent intake of Iron
- Haemolysis
- Renal disease
- Liver disease
- Alcohol misuse
- Metabolic syndrome, obesity, diabetes
- Malignancy
- Auto-immune disease
- Hereditary Haemochromatosis

Although raised ferritin will be due to non-iron overload causes in up to 90% of patients, it is important to consider genetic haemochromatosis to avoid iron overload and end-organ damage by delay in diagnosis.



Referral Criteria

The following patients should be referred to HEPATOLOGY

- Confirmed (fasting) Biochemical testing meeting the below criteria with or without abnormal HFE gene

	TSAT	Ferritin
Males	>50%	>300 ug/L
Females	>45%	>200 ug/L

- Patients with C282Y homozygosity or C282Y/H63D compound heterozygosity of HFE with a ferritin above EASL cut off and/or abnormal LFTs

Patients with C282Y homozygosity or C282Y/H63D compound heterozygosity and normal ferritin (usually screened relatives of index case) do not require referral, but should have an annual ferritin measurement. If ferritin is raised, exclude concurrent infection/inflammation and refer if ferritin remains raised after 4-6 weeks.

Ferritin >10,000 ug/L

This is an unusual finding in general practice and can be seen in Still's disease, macrophage haemophagocytic syndrome or massive blood transfusion. A significantly raised ferritin should warrant consultant haematologist referral.

Contacts

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Patient information sheet

Genetic Haemochromatosis A Patient centred guideline 1st Ed 2025 Haemochromatosis UK
[Here](#)

References

1. Diagnosis and therapy of genetic haemochromatosis. British Journal of Haematology 2018: <https://onlinelibrary.wiley.com/doi/pdf/10.1111/bjh.15164>
2. EASL Clinical Practice guidelines on Haemochromatosis J Hepatol. 2022 Aug;77(2): 479 - 502
3. Diagnosis and therapy of genetic haemochromatosis (review and 2017 update) Br J Haematol. 2018 May;181(3):293-303.

Document title	Hyperferritinaemia		
Revision	4	Document Reference	CHISCP50
Reason for review	Routine review		
Review history	Version/Date	Authors	Reason for review
	V1 Sept 2018	Dr John Monaghan, Mrs H Seddon, Dr Adam Lawson	New Document
	V2 July 2021	Dr A Austin, Dr K Dear, Dr A Lawson, Dr P Blackwell, Mrs H Seddon	Routine review
	V3 Jan 2024	Dr P Blackwell, Mrs H Seddon	Routine review
Approval	Date	Name(s)	Designation
Secondary Care UHDB	24/03/2026	Dr Claire Grant	Consultant Hepatologist
Secondary Care CRH	27/02/2026	Dr K Dear	Consultant Gastroenterologist
Primary Care	08/05/2026	Dr P Blackwell	GP
Laboratory (Derbyshire Pathology)	16/06/2026	Ms S Knowles	Consultant Clinical Scientist